

إسم المريض رباعياً : ايه عمرو ابراهيم حسن
المحافظة : القاهرة
رقم البطاقة : 29710240100084 رقم التليفون : 01212442721
العنوان : دار السلام
العمر(تاريخ الميلاد) : 24-10-1997 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 19-09-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي
الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157]
التعليق على التشخيص : -----

الاسم: د/محمد كيلاني الوظيفة: _____
 الاسم: د/ندى حسن الوظيفة: _____
 الاسم: د/نرمين رمضان الوظيفة: _____
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 جهة العلاج :
 نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

--

Name: ايه عمرو ابراهيم حسن National ID: 29710240100084	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



أبوه

عمرو إبراهيم حسن

أش بشير نهابة ش مطر

دار السلام - القاهرة

١٩٩٧/١



٢٩٧ ١٠ ٢٤ ٠١ ٠٠٠ ٨٤

K03299746



٢٠٢٤/٠٦

٢٩٧١٠٢٤٠١٠٠٠٨٤

ربة منزل



أنشى مسلمة
محمد عبد المجيد عبد الفتاح أحمد
البطاقة سارية حتى ٢٠٣١/٠٦/٠١





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : ايه عمرو ابراهيم حسن

Sex:

3- Medical number:
Female

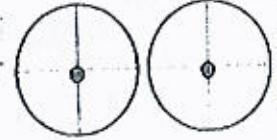
83210578
Old Med :

Old Med :
1742-2024

2-ID number: 29710240100084

Patient Age : 27 Y

MDT



Family history

Family History

Phone

Phone
01212442721

BMI

BMI

REASON OF MDT

REASON OF MDT
New

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date Of MDT
2024-06-24 ☐

Co morbidity

Co Morbidity

Menopausal
status

Menopausal Status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :

ct chest: multilevel vertebral body lytic lesions for bone scan
bone scan: abnormal active focal radiotracer uptakes are
seen

BX
IDC

xx COMMENT

gli with focal DCIS

ER
7/8

ER COMMENT

PR
0/8

PR COMMENT:

Her 2 Neu
0

Her 2 Neu COMMENT

SISH

SH COMMENT :

Ki67
25

T
pT2

N
N3

M
1 bone

Sonomammography

PATIENT HISTORY
MBC to bone

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

For 1st lone Zoladex - FEMARA Zometa every 3 months (consider KISQALI if affordable)

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

د/ لؤي قاسم

RADIOLOGY

RADIOLOGY

د/ رهاب البكري

SURGERY

SURGERY

د/ محمد سلامة

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

Nuclear Medicine Unit

Patient Name: Aya Amr Ibraheem

Date: 4/1/2025

Referring physician: Prof.

^{99m}Tc -MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

Report:

- Compared to the previous bone scan study , dated 6/9/2024 , the current scan revealed , no significant appreciable changes could be seen , since the last study .
- Study defined stationary course of the previously seen active osseous lesions are seen at multilevel , spinal vertebrae , (CV 2 , Dv 10 , 11 , LV 3 , Lv5) for follow up .
- Osteoarthritic changes are seen at both shoulders , both knees .
- Both kidneys are normally seen.

Conclusion:

Compared to the previous bone scan study dated 6/9/2024 , the current scan revealed , stationary course of the disease with no newly developed active osseous lesions as mentioned .

Nuclear Medicine Unit

DR . Sherien Mahdy, MD

DR/ Mokamed Emam

Patient Name : الأستاذة / ايه عمرو إبراهيم حسن
Patient No. : 29710240100084
Path No. : SR-718/24
Sex : Female
Age : 26 Y
Referring Doc :

Request No. : 13003485
Request Date : 04/06/2024
Verification Date : 04/06/2024
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass
Nature of Specimen: Slide revision.

Gross Examination

Fourteen slides labeled by (465), four slides labeled by (208) and 5 slides labeled by (431/24) were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade II.
- * DCIS: Focal.
- * Angiolysis: Couldn't be detected in the received biopsy
- * Perineural invasion: Positive.
- * Skin involvement: Free.
- * Lymph nodes: Positive (18/18).
- * IPT:
- ER: Moderate nuclear reaction in 80% tumor cells (score 7/8)
- PR: No nuclear reaction in tumor cells (score 0/8)
- Her2: No membrane reaction in tumor cells (score 0)
- Ki67: 25% nuclear reaction of tumor cells.

Diagnosis:

Right breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- Free skin (nipple).
- Positive lymph nodes (18/18).
- Positive perineural invasion.
IPT:

Verified by :

Printing Date : 05/06/2024 14:31:22

Page 1 of 2

Printing By : marwa mohamed youss





Patient Name: Aya Amr Ibrahim Hassan	Date : 22/10/2024
Exam : C.T CHEST WITH CONTRAST	Age : 27 Year(s)
Medical ID: 1742/2024	Pat. ID: 54495

**POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN, AND PELVIS: -**

Findings:

Compared to last CT study dated 2/06/2024

I Chest:

- Evidence of right mastectomy with still clear operative bed.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. Still no focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs. IUD seen in uterus.
- Gall bladder stone is seen.
- No abdominal or pelvic masses or collections.

The skeletal survey:

Patient Name: Aya Amr Ibrahim Hassan	Date : 22/10/2024
Exam : C.T CHEST WITH CONTRAST	Age : 27 Year(s)
Medical ID: 1742/2024	Pat. ID: 54495

- Still seen multilevel lumbar and dorsal vertebral bodies bony deposit with sclerotic texture in today's study denoting treatment response.

Opinion:

- Follow up status showing
- Sclerotic transformation of osseous lesions.
- Otherwise, stationary course since last study.

Dr. Shalmaa Megahed, MSc

Prof. Menna Talla Hanafy, M.D.

Nuclear Medicine Unit

Patient Name: Aya Amr Ibraheem

Date: 4/1/2025

Referring physician: Prof.

^{99m}Tc -MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

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- Study defined stationary course of the previously seen active osseous lesions are seen at multilevel, spinal vertebrae, (CV 2, Dv 10, 11, LV 3, Lv5) for follow up.
- Osteoarthritic changes are seen at both shoulders, both knees.
- Both kidneys are normally seen.

Conclusion:

Compared to the previous bone scan study dated 6/9/2024, the current scan revealed, stationary course of the disease with no newly developed active osseous lesions as mentioned.

Nuclear Medicine Unit

DR. Sherien Mahdy, MD

DR/ Mokamed Emam

Patient Name: Aya Amr Ibrahim Hassan	Date : 25/06/2025
Exam : C.T CHEST	Age : 27 Year(s)
Medical ID: 1742/2024	Pat. ID: 54495

**NON -CONTRAST CT SCAN OF THE CHEST ,
ABDOMEN AND PELVIS : -**

Findings:

- Clear both lung fields ,
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal appearance of the thoracic aorta and the great vessels.
- **Average size liver showing homogenous density. No gross focal lesions**
- **Average size spleen , no focal lesions.**
- ***Average size gall bladder ,harboring small stone 3mm.**
- *No dilated intra or extra-hepatic bile ducts .
- *Urinary bladder shows no masses or stones.
- *Normal C.T. appearance of the pancreas , adrenal glands, both kidneys ,bowel loops & other pelvic organs.
- *No retroperitoneal lymphadenopathy .
- *No ascites .
- **The examined bones show mixed lytic , sclerotic lesions involving D6, D9, D10, D11 ,L1, L2, L3 and L5 vertebrae as well as right D9 pedicle and left iliac bone**
- **Opinion:**

Known case of breast cancer

- **Bony metastatic deposits as mentioned as compred to mentioned data,,,,progressive course**

DR/Reham khedr...MD

Signature Dr. Mohamed Hassan

26/06/2025 7:02 PM